CITY GENERAL HOSPITAL

DEPARTMENT OF INTERNAL MEDICINE | RESPIRATORY UNIT

12 Health Avenue, New Delhi, India | Tel: 011-4000-0000

PATIENT HEALTH REPORT

For OCR / document-parsing testing only — not a real medical record

PATIENT DETAILS

Patient Name: SAMPLE PATIENT

Patient ID: DEMO-PT-2026-0417

Age / Sex: 58 years / Male

Date of Visit: 03-Nov-2026

Department: Emergency Medicine

Report No.: DEMO-PNEU-0001

Referring Clinician: Dr. A. Sharma (Fictional)

Status: Sample / Synthetic

PRESENTING COMPLAINTS & HISTORY

Chief complaints: Fever, productive cough, and shortness of breath for 4 days.

Associated symptoms: Chills, fatigue, right-sided chest discomfort on coughing.

Relevant history: No known drug allergies reported in this sample.

Examination: Alert and oriented. Crackles heard over the right lower lung field.

VITAL SIGNS

Parameter	Observed Value	Reference / Note	Unit
Temperature	38.6	36.1–37.2	°C
Pulse	108	60–100	beats/min
Respiratory rate	26	12–20	breaths/min
Blood pressure	118/76	Typical adult range	mmHg
SpO_2 (room air)	91	Usually ≥ 95	%

LABORATORY INVESTIGATIONS

Test	Result	Reference / Note	Unit
Hemoglobin	13.4	13.0–17.0	g/dL
Total WBC count	15,200	4,000–11,000	cells/µL
Neutrophils	82	40–75	%
CRP	68	< 5	mg/L

RADIOLOGY

Chest X-ray (PA view):

Patchy air-space opacity in the right lower zone, suggestive of infective consolidation.

No pleural effusion or pneumothorax identified in this sample report.

Impression: Findings are compatible with right lower lobe pneumonia.

ASSESSMENT & PLAN

Provisional diagnosis: Community-acquired pneumonia (right lower lobe).

Plan: Clinical review, oxygen saturation monitoring, hydration, and clinician-directed treatment. Antibiotic choice and duration to be determined by the treating physician.

Return urgently for worsening breathlessness, confusion, bluish lips, or falling SpO_2.

Prepared: 03-Nov-2026 16:00 IST

Authorized Signatory: _____

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SYNTHETIC SAMPLE